

Johnson & Johnson (JNJ)

KOL Survey Bullish on Icotyde Potential and Positioning in Psoriasis

JNJ 12m Price Target: **\$275.00** Price: **\$267.24** Upside: **2.9%**

JNJ's recently launched oral psoriasis pill, Icotyde, represents a key new product cycle in the company's immunology franchise that management has framed as having the potential to be one of the company's largest products ever, making it a crucial piece of the puzzle for the company's stated goal toward double digit revenue growth by the end of the decade. Following FDA approval in March for moderate-to-severe plaque psoriasis, on the 1Q26 earnings call in mid-April, JNJ highlighted positive early launch indicators, noting 1,500 patients with prescriptions had been written with over 1,000 unique prescribers; subsequently, on June 9th at our healthcare conference, the company disclosed that prescriber count had grown to ~4,500, representing a steep ramp. The company has said that IQVIA is not yet capturing all channels, suggesting public script data may be underestimating actual demand. JNJ's 2Q26 earnings results next week (see our preview here) represent the first full quarter of Icotyde sales, and we expect management to provide updated launch metrics, although not yet break out product level sales. In addition to psoriasis, Icotyde is also being developed in psoriatic arthritis (PsA), ulcerative colitis (UC) and Crohn's disease (CD) and management has previously said that Icotyde will be a meaningful revenue driver by 2027, at an enterprise level.

To assess Icotyde's launch progress and longer-term positioning, we conducted a proprietary survey last week of 26 dermatologists. We note the following key takeaways:

1. Dermatologists expect Icotyde prescriptions for moderate-to-severe psoriasis patients to ramp significantly — from ~2-3% currently to ~8-10% in 1 year, and to 15-20% over 5 years.
2. Icotyde is seen as most apt for patients new to branded systemics or switches from other oral brands (e.g. AMGN's Otezla and BMY's Sotyktu). While switching from other branded

BUY ^{CL}

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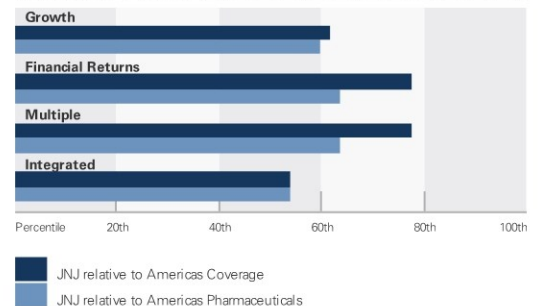
Key Data

Market cap: \$650.4bn
Enterprise value: \$678.2bn
3m ADTV: \$1.9bn
United States
Americas Pharmaceuticals
M&A Rank: 3

GS Forecast

	12/24	12/25E	12/26E	12/27E
Revenue (\$ mn)	88,821.0	94,193.0	101,136.5	110,064.0
EBITDA (\$ mn)	33,805.0	38,814.0	41,918.4	46,602.6
EBIT (\$ mn)	26,466.0	30,358.0	33,826.7	38,302.3
EPS (\$)	9.98	10.77	11.60	13.38
P/E (X)	15.6	24.8	23.0	20.0
EV/EBITDA (X)	11.5	17.5	16.2	14.2
FCF yield (%)	5.2	3.1	2.1	4.7
Dividend yield (%)	3.1	1.9	1.9	2.0
Net debt/EBITDA (X)	0.4	0.7	0.7	0.3
	6/25	9/25	12/25E	--
EPS (\$)	2.77	2.80	2.46	--

GS Factor Profile



Source: Company data, Goldman Sachs Research estimates.
See disclosures for details.

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Johnson & Johnson (JNJ)

Rating since Apr 8, 2025

Ratios & Valuation

	12/24	12/25E	12/26E	12/27E
P/E (X)	15.6	24.8	23.0	20.0
EV/EBITDA (X)	11.5	17.5	16.2	14.2
EV/sales (X)	4.4	7.2	6.7	6.0
FCF yield (%)	5.2	3.1	2.1	4.7
EV/DACF (X)	19.0	18.8	23.8	19.3
CROCI (%)	16.5	25.3	17.6	20.0
ROE (%)	34.6	34.3	33.2	34.0
Net debt/EBITDA (X)	0.4	0.7	0.7	0.3
Net debt/equity (%)	16.9	34.1	32.3	12.8
Interest cover (X)	—	—	91.3	—
Inventory days	191.9	190.9	189.0	185.1
Receivable days	61.1	62.0	63.5	62.4
Days payable outstanding	162.0	159.9	153.4	143.7

Growth & Margins (%)

	12/24	12/25E	12/26E	12/27E
Total revenue growth	4.3	6.0	7.4	8.8
EBITDA growth	(3.3)	14.8	8.0	11.2
EPS growth	0.7	8.0	7.7	15.3
DPS growth	2.3	3.3	3.0	3.0
Gross margin	74.7	73.0	72.7	73.6
EBIT margin	29.8	32.2	33.4	34.8

Price Performance



Source: FactSet. Price as of 7 Jul 2026 close.

Income Statement (\$ mn)

	12/24	12/25E	12/26E	12/27E
Total revenue	88,821.0	94,193.0	101,136.5	110,064.0
Cost of goods sold	(22,468.0)	(25,457.0)	(27,564.8)	(29,056.9)
SG&A	(22,853.0)	(23,676.0)	(25,161.3)	(26,415.4)
R&D	(17,034.0)	(14,702.0)	(14,583.6)	(16,289.5)
Other operating inc./ (exp.)	—	—	—	—
EBITDA	33,805.0	38,814.0	41,918.4	46,602.6
Depreciation & amortization	(7,339.0)	(8,456.0)	(8,091.6)	(8,300.3)
EBIT	26,466.0	30,358.0	33,826.7	38,302.3
Net interest inc./ (exp.)	577.0	85.0	(370.6)	206.8
Income/ (loss) from associates	—	—	—	—
Pre-tax profit	28,979.0	31,681.0	34,556.1	39,609.1
Provision for taxes	(4,737.0)	(5,466.0)	(6,179.7)	(7,083.4)
Minority interest	—	—	—	—
Preferred dividends	—	—	—	—
Net inc. (pre-exceptionals)	24,242.0	26,215.0	28,376.4	32,525.7
Net inc. (post-exceptionals)	15,891.0	26,804.0	22,860.4	27,509.7
EPS (basic, pre-exception) (\$)	9.98	10.77	11.63	13.38
EPS (diluted, pre-exception) (\$)	9.98	10.77	11.60	13.38
EPS (ex-ESO exp., dil.) (\$)	--	--	--	--
DPS (\$)	4.81	4.97	5.12	5.27
Div. payout ratio (%)	48.2	46.1	44.0	39.4
Wtd avg shares out. (basic) (mn)	2,428.9	2,433.1	2,439.0	2,430.4
Wtd avg shares out. (diluted) (mn)	2,428.9	2,433.1	2,445.2	2,430.4

Balance Sheet (\$ mn)

	12/24	12/25E	12/26E	12/27E
Cash & cash equivalents	24,522.0	20,102.0	17,563.2	31,672.8
Accounts receivable	14,842.0	17,178.0	18,010.6	19,600.4
Inventory	12,444.0	14,191.0	14,348.8	15,125.5
Other current assets	4,085.0	4,153.0	4,256.9	4,412.5
Total current assets	55,893.0	55,624.0	54,179.5	70,811.3
Net PP&E	20,518.0	23,169.0	34,247.9	34,900.4
Net intangibles	37,618.0	48,772.0	44,772.0	40,772.0
Total investments	0.0	0.0	0.0	0.0
Other long-term assets	66,075.0	71,645.0	74,831.3	76,877.6
Total assets	180,104.0	199,210.0	208,030.6	223,361.3
Accounts payable	10,311.0	11,991.0	11,177.0	11,702.4
Short-term debt	5,983.0	8,495.0	8,767.0	8,806.0
Current lease liabilities	—	—	—	—
Other current liabilities	34,027.0	33,640.0	36,119.8	39,308.2
Total current liabilities	50,321.0	54,126.0	56,063.8	59,816.5
Long-term debt	30,651.0	39,438.0	37,680.6	35,923.2
Non-current lease liabilities	—	—	—	—
Other long-term liabilities	27,642.0	24,102.0	24,867.3	25,961.8
Total long-term liabilities	58,293.0	63,540.0	62,547.9	61,885.0
Total liabilities	108,614.0	117,666.0	118,611.7	121,701.5
Preferred shares	—	—	—	—
Total common equity	71,490.0	81,544.0	89,418.9	101,659.8
Minority interest	—	—	—	—
Total liabilities & equity	180,104.0	199,210.0	208,030.6	223,361.3
BVPS (\$)	29.43	33.52	36.57	41.83

Cash Flow (\$ mn)

	12/24	12/25E	12/26E	12/27E
Net income	15,891.0	26,804.0	22,860.4	27,509.7
D&A add-back	7,339.0	7,503.0	8,091.6	8,300.3
Minority interest add-back	—	—	—	—
Net (inc)/dec working capital	3,013.0	(11,364.0)	571.5	1,191.6
Others	(2,266.0)	1,788.0	(2,703.2)	(1,303.7)
Cash flow from operations	23,977.0	24,731.0	28,820.3	35,697.9
Capital expenditures	(4,424.0)	(4,832.0)	(15,170.5)	(4,952.9)
Acquisitions	(15,146.0)	(17,541.0)	—	—
Divestitures	—	—	—	—
Others	320.0	(1,239.0)	(958.0)	(1,242.8)
Cash flow from investing	(19,250.0)	(23,612.0)	(16,128.5)	(6,195.7)
Dividends paid	(11,823.0)	(12,381.0)	(12,485.5)	(12,768.9)
Share issuance/(repurchase)	(2,432.0)	(5,953.0)	(2,500.0)	(2,500.0)
Inc/(dec) in debt	11,021.0	9,637.0	(1,485.4)	(1,718.4)
Others	102.0	3,158.0	1,240.3	1,594.6
Cash flow from financing	(3,132.0)	(5,539.0)	(15,230.6)	(15,392.7)
Total cash flow	1,595.0	(4,420.0)	(2,538.8)	14,109.6
Free cash flow	19,553.0	19,899.0	13,649.8	30,745.1
Free cash flow per share (basic) (\$)	8.05	8.18	5.60	12.65

Source: Company data, Goldman Sachs Research estimates.

orals to Icotyde is expected, not as much switching is expected from injectable IL-23 or IL-17 agents. Injectable IL-23 agents (e.g. ABBV's Skyrizi and JNJ's Tremfya) are still preferred for ~50% of new patients.

3. Key factors for prescribing are efficacy and oral administration, and Icotyde's fasting requirement has not been a barrier.

Overall, we see the survey results as consistent with JNJ (and ABBV) messaging that Icotyde is likely to expand the psoriasis treatment market and not detract from IL-23 injectables (such as Tremfya and Skyrizi). Notably, if Icotyde achieves the 15-20% market share indicated by the KOLs in our survey, this could support \$10bn+ peak revenue (vs. GSe \$7.7bn/ Visible Alpha consensus data \$7.1bn), with potential upside from other indications such as PsA, UC and CD.

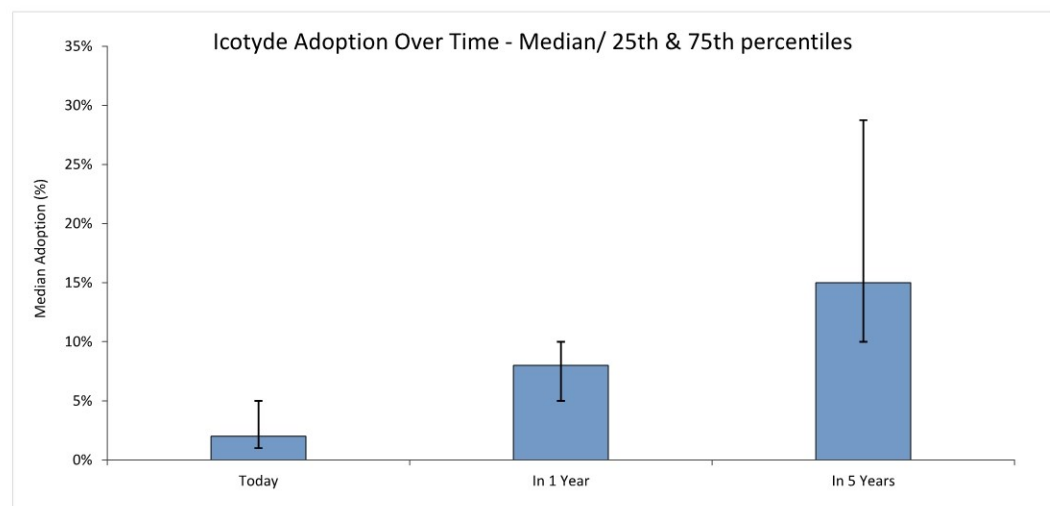
Details within.

Detailed Icotyde KOL Survey Results

We surveyed 26 dermatologists who each treat a minimum of 100 moderate-to-severe psoriasis patients and have prescribed multiple branded systemics in the past year.

Q1. What % of your moderate/severe psoriasis patients have you prescribed Icotyde as of today? What % do you anticipate in 1 year? In 5 years?

Exhibit 1: Dermatologists Expect to Ramp Up Icotyde Prescribing for Moderate-to-Severe Psoriasis Patients

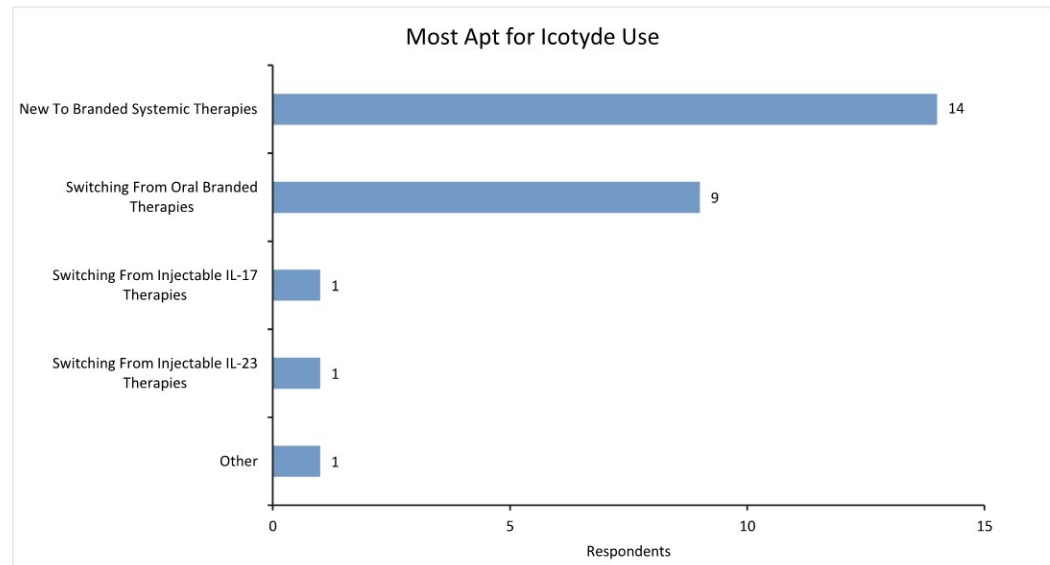


Source: Data compiled by Goldman Sachs Global Investment Research, Goldman Sachs Global Investment Research

Dermatologists indicated they have prescribed Icotyde to an average/median percent of their eligible populations today of 3%/2%, and that is expected to grow to 10%/8% in 1 year and 19%/15% in 5 years. Interestingly, observing the 25th and 75th percentiles, while the 25th percentile followed a similar ramp (from 1% to 5% to 10%), the 75th percentile indicated a skew up to 29% in 5 years — this suggests to us that a number of physicians see Icotyde being utilized more heavily over time, representing potential upside.

Q2. What existing population of moderate/severe psoriasis patients do you see as being most apt to using Icotyde?

Exhibit 2: Icotyde Patients Primarily Expected to be Either New to Branded Systemic or Switching from Oral Branded

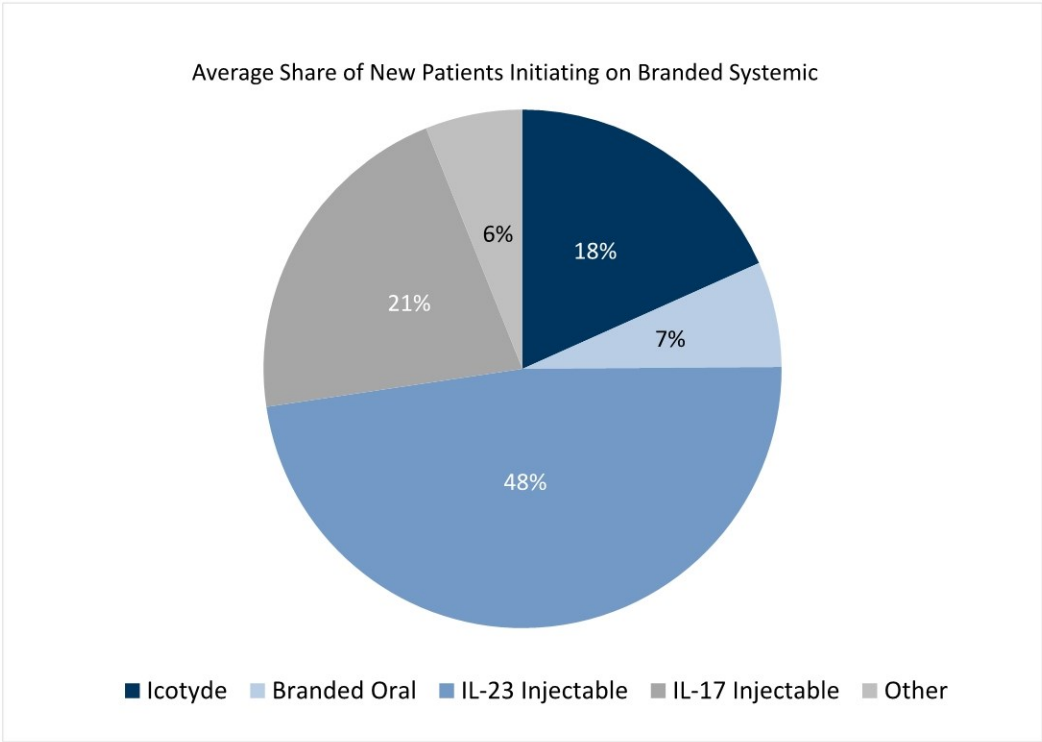


Source: Data compiled by Goldman Sachs Global Investment Research, Goldman Sachs Global Investment Research

We see these survey responses as consistent with JNJ's commentary of Icotyde vis a vis Tremfya (and ABBV's commentary vis a vis Skyrizi). JNJ has noted that Icotyde is positioned as a first-line systemic therapy, apt to capture patients transitioning from topical treatments or existing oral therapies (e.g. Otezla or Sotyktu) who are hesitant to start injectable biologics. Management believes that the introduction of an oral IL-23 peptide will expand the advanced systemic market rather than take Tremfya's share. At our Global Healthcare Conference, JNJ noted that ~60% of early Icotyde patients were systemic treatment-naïve and ~25% of patients were switching from existing oral treatments, directionally consistent with our survey.

Q3. Among NEW patients initiating on a branded systemic agent, what percent would you anticipate prescribing the following:

Exhibit 3: IL-23 and IL-17 Injectables are Still Favored Overall for Moderate-to-Severe Psoriasis Patients

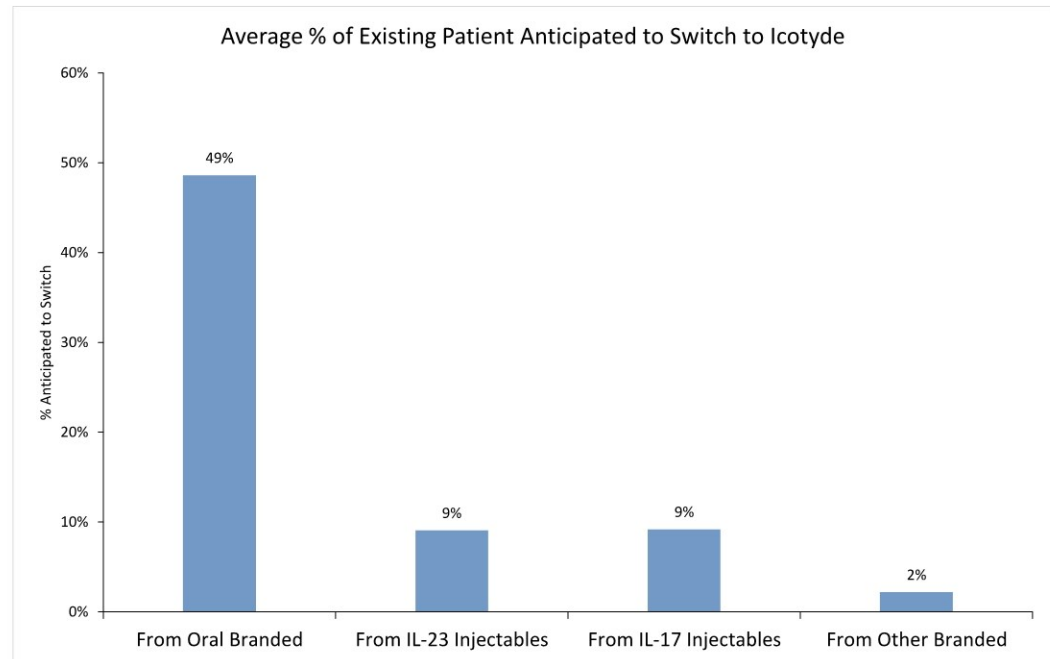


Source: Data compiled by Goldman Sachs Global Investment Research

Consistent with responses to the prior question, injectable IL-23 agents (such as Tremfya and Skyrizi) were indicated to be favored for roughly half of new patients on average. Our survey indicated dermatologists on average slightly leaned towards injectable IL-17 as the next choice, although closely followed by Icotyde.

Q4. Among EXISTING patients, what % do you anticipate would switch to Icotyde... from oral branded therapies? From branded injectables?

Exhibit 4: Significant Switching is Expected from Other Oral Branded Options, but Not from IL-23 or IL-17 Injectables

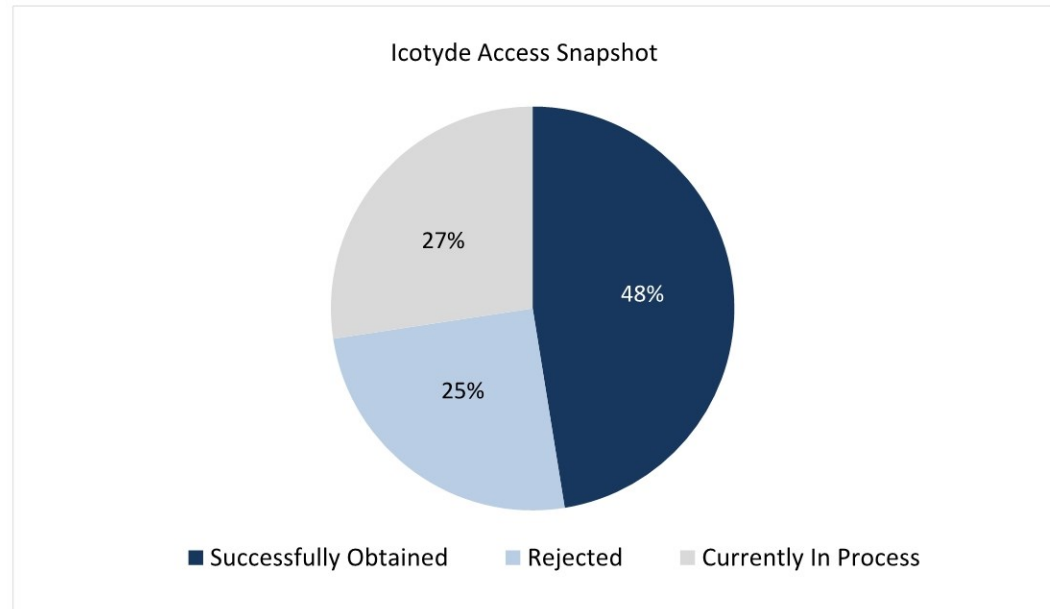


Source: Data compiled by Goldman Sachs Global Investment Research

While roughly half of patients on oral branded therapies (e.g. Otezla and Sotyktu) are anticipated to switch to Icotyde (which seems logical to us given higher efficacy with similar dosing), only a single digit percent of patients on existing injectable IL-23 or IL-17 agents would be anticipated to switch (also logical, as we understand physicians are hesitant to switch patients who are stable on existing therapy, and the injectables generally offer convenient profiles with monthly to quarterly dosing).

Q5. What have been the insurance coverage/ access results for your patients that have you attempted to prescribe Icotyde?

Exhibit 5: Dermatologists Suggest the Access Success Rate is 2:1, Albeit Early Days with Many Still in Process

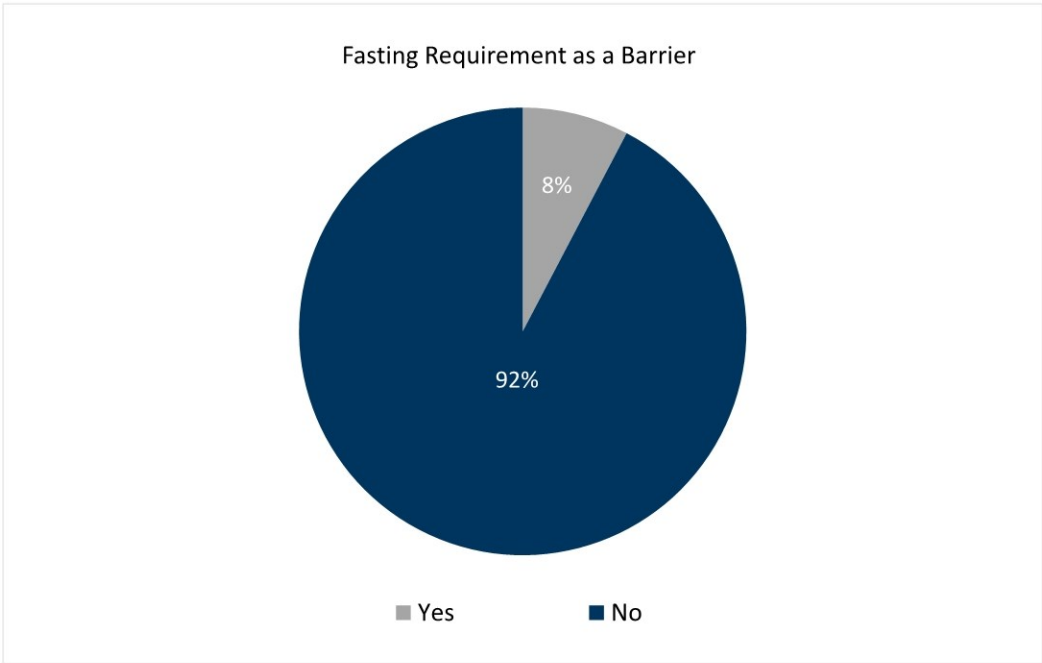


Source: Data compiled by Goldman Sachs Global Investment Research

We view the survey indication of roughly half of prescribed patients successfully accessing Icotyde as encouraging, representing a 2:1 success ratio, with many prescriptions still in progress. We see this as likely to improve further as the launch matures and physicians/payors gain more experience and become more familiar with navigating the logistics.

Q6. Has the 30-minute fasting dosing been a barrier to prescribing?

Exhibit 6: The 30-minute Fasting Post Dosing Has Overwhelmingly Not Been a Barrier

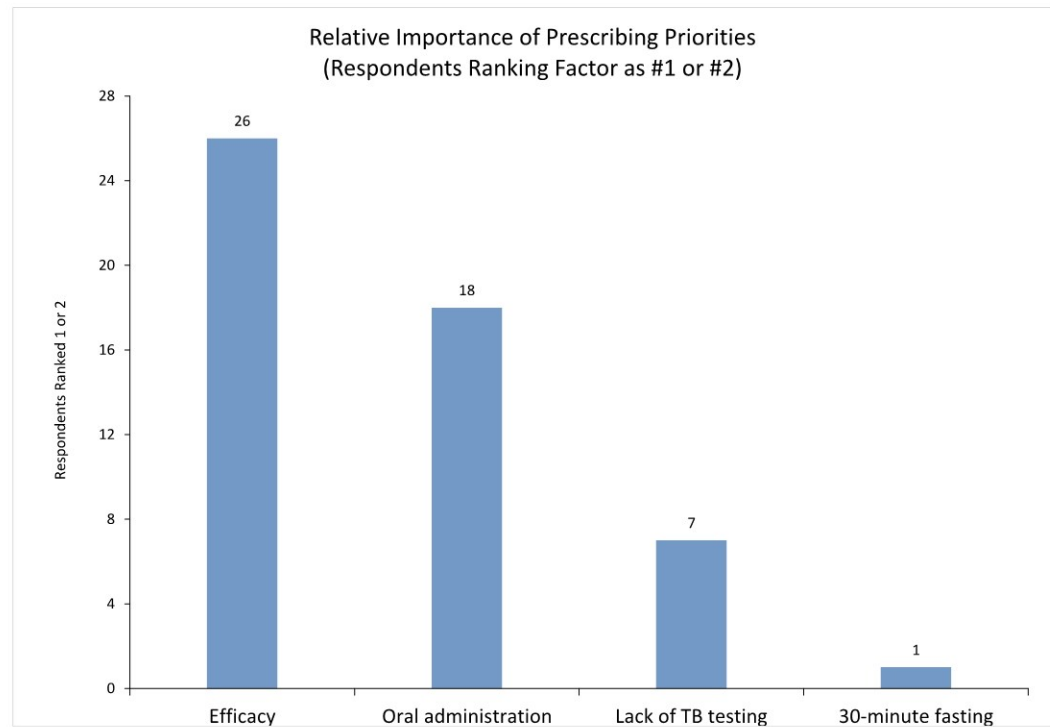


Source: Data compiled by Goldman Sachs Global Investment Research

Encouragingly, the vast majority of physicians do not consider the food effect/ fasting element to be a barrier to prescribing Icotyde.

Q7. Rank the relative importance of the following factors you would consider for prescribing:

Exhibit 7: Efficacy and Oral Administration are the Top Drivers of Icotyde Prescribing



Source: Data compiled by Goldman Sachs Global Investment Research

We see the survey indication of dermatologists' prescribing factors as logical and consistent with the other survey responses, with efficacy and oral administration as the top factors. From an efficacy perspective, Icotyde places above other existing oral therapies, although shy of injectable IL-23s — however is able to offer oral differentiation. Lack of TB testing was seen as a less important factor, and 30-minute fasting was seen as least important.

Valuation and Risks

Valuation: We arrive at our 12-month price target of \$275 based on a 20x P/E multiple on our Q5-Q8 EPS.

Key downside risks: The ramp of new products could be slower than we expect, resulting in less revenue to replenish declining products. Further uncertainty related to the Talc litigation could result in higher-than-expected payouts and hamper multiples. A more-difficult-to-navigate external environment related to macro and tariffs could create downside to our estimates.

Disclosure Appendix

Reg AC

We, Asad Haider, CFA, Nick Jennings and Jeff Su, hereby certify that all of the views expressed in this report accurately reflect our personal views about the subject company or companies and its or their securities. We also certify that no part of our compensation was, is or will be, directly or indirectly, related to the specific recommendations or views expressed in this report.

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Financial Returns and Multiple use the Goldman Sachs analyst forecasts at the fiscal year-end at least three quarters in the future. Growth uses inputs for the fiscal year at least seven quarters in the future compared with the year at least three quarters in the future (on a per-share basis for all metrics).

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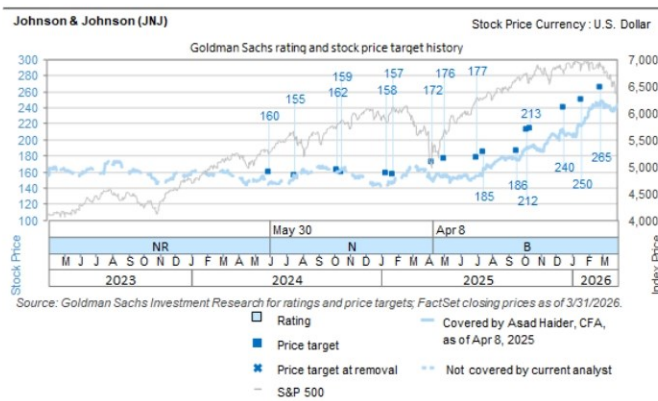
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	Rating Distribution				Investment Banking Relationships		
	Buy	Hold	Sell		Buy	Hold	Sell
Global	50%	34%	16%		65%	60%	45%

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Target price history table(s)

Johnson & Johnson (JNJ)

Date of report	Target price (\$)	Closing price (\$)
15-Apr-26	275.00	238.67
27-Feb-26	265.00	248.43
22-Jan-26	250.00	218.49
18-Dec-25	240.00	208.31
15-Oct-25	213.00	191.17
08-Oct-25	212.00	189.69
19-Sep-25	186.00	176.19
17-Jul-25	185.00	162.98
03-Jul-25	177.00	156.01
02-May-25	176.00	156.12
08-Apr-25	172.00	150.00
23-Jan-25	157.00	146.64
10-Jan-25	158.00	142.06
16-Oct-24	159.00	164.28
08-Oct-24	162.00	159.69
18-Jul-24	155.00	155.42
30-May-24	160.00	145.28

Price targets shown in table(s) are unadjusted for corporate actions.

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